APEX HEALTHCARE MOLECULAR LABS

NABH & NABL ACCREDITED DIAGNOSTIC MOLECULAR PATHOLOGY LABORATORY
108 Medical Center Boulevard, Tech Health City • Phone: (800) 555-LABS

Patient Name: Emily Watson	**Patient ID:** PT-2026-4432	**Age / Gender:** 28 Years / Female
Referred By: Dr. Anita Sharma, MD	**Collection Date:** 2026-09-18	**Report Date:** 2026-09-18

LABORATORY TEST RESULTS

Test Name / Parameter	Result	Units	Normal Reference Interval	Status Flag
Hemoglobin (Hb)	**9.2**	g/dL	12.0 - 15.5	**LOW**
Red Blood Cell Count (RBC)	**3.4**	x10^6/uL	3.9 - 5.2	**LOW**
Hematocrit (PCV)	**28.5**	%	36.0 - 46.0	**LOW**
Mean Corpuscular Volume (MCV)	**71.4**	fL	80.0 - 100.0	**LOW**
Mean Corpuscular Hemoglobin (MCH)	**23.5**	pg	27.0 - 33.0	**LOW**
Mean Corpuscular Hemoglobin Conc. (MCHC)	**30.1**	g/dL	32.0 - 36.0	**LOW**
Red Cell Distribution Width (RDW)	**17.2**	%	11.5 - 14.5	**HIGH**
White Blood Cell Count (WBC / TLC)	6400	/cumm	4000 - 11000	NORMAL
Platelet Count	295	x10^3/uL	150 - 450	NORMAL

CLINICAL COMMENTS & INTERPRETATION

- Moderate microcytic hypochromic anemia demonstrated by low Hemoglobin (9.2 g/dL), low MCV (71.4 fL), and low MCH.
- Elevated RDW (17.2%) suggests anisocytosis, characteristic of iron deficiency anemia.
- Total leukocyte and platelet counts are within normal physiological thresholds.
- Recommended follow-up: Serum Ferritin, Total Iron Binding Capacity (TIBC), and dietary iron evaluation.

Electronically verified by Dr. Sarah Jenkins, MD (Consultant Clinical Pathologist). Medical Disclaimer: These diagnostic test results must be interpreted clinically by the treating medical doctor.